SMART EMERGENCY GUIDE — BOARD-PREP SUPPLEMENT · 01.11.2026

Emergency Medicine High-Yield Reference
Non-Pharmacologic Core

Clinical patterns, decision rules, physical signs, lab thresholds, diagnostic criteria, red flags, mnemonics and exam pearls — the recognition-and-reasoning layer of EM, independent of drug dosing, ECG atlas, imaging atlas, OSCE stations, oral-board cases and image banks.

مرجع الطوارئ السريع — الركيزة المعرفية غير الدوائية

INCLUDED IN THIS DOCUMENT

- Classic triads & pathognomonic patterns
- Clinical decision rules & scoring systems
- Critical time windows
- Physical exam signs & maneuvers
- Toxidrome recognition (clinical pattern only)
- Lab values & critical thresholds
- Diagnostic criteria & definitions
- Red flags / must-not-miss
- Mnemonics
- Exam pearls & traps
- Communication frameworks

DELIBERATELY EXCLUDED (per request)

- Medication names, doses & antidotes
- ECG pattern atlas
- Radiology / imaging atlas
- OSCE station checklists
- Oral-board case simulations
- Clinical image banks

This is a curated **high-yield core**, not an exhaustive extraction of every triad, rule, sign and mnemonic in emergency medicine — the source brief itself asks for examination-oriented prioritization (Priority 1 / 2 / 3). Items below are the ones most commonly tested on Arab Board and ABEM written/oral exams. Values that are assay- or guideline-dependent are flagged for local verification rather than stated as universal.

هاد ملخص عالي التركيز (High-Yield) مو استخراج شامل لكل شي موجود بالطب، لأنه حتى الوثيقة الأصلية طلبت ترتيب حسب الأولوية. القيم يلي بتختلف حسب المختبر أو الدليل الإرشادي محددة بإشارة تنبيه بدل ما تُكتب كأنها ثابتة بكل مكان.

01 Classic Triads & Pathognomonic Patterns

تشكيلات سريرية كلاسيكية — لما تجتمع هاي العلامات مع بعض بتدل بشكل شبه مؤكد على تشخيص معيّن.

Pattern	Components	Points to	شرح مبسّط
Beck's triad	JVD · hypotension · muffled heart sounds	Cardiac tamponade	احتقان أوردة الرقبة + هبوط ضغط + أصوات قلب مكتومة ← انضغاط القلب بسائل داخل التامور، يحتاج بزل عاجل
Cushing's triad	Hypertension · bradycardia · irregular respirations	Raised ICP / impending herniation	ارتفاع ضغط + بطء نبض + تنفس غير منتظم ← علامة متأخرة وخطيرة على ارتفاع الضغط داخل الجمجمة، غالبًا قبل الفتق الدماغي مباشرة
Charcot's triad	Fever · jaundice · RUQ pain	Ascending cholangitis	حمى + يرقان + ألم بالربع العلوي الأيمن ← التهاب القنوات الصفراوية الصاعد، غالبًا بسبب انسداد حصوي
Reynolds' pentad	Charcot's triad + hypotension + altered mental status	Suppurative cholangitis (surgical emergency)	نفس أعراض شاركو + هبوط ضغط + تغيّر بالوعي ← حالة إنتانية حرجة تحتاج تصريف عاجل للقنوات الصفراوية
Virchow's triad	Venous stasis · endothelial injury · hypercoagulability	Framework for thrombosis (DVT/PE) risk	ركود وريدي + إصابة بطانة الوعاء + فرط تخثر ← الإطار الكلاسيكي لفهم أسباب تكوّن الجلطات، مو تشخيص بحد ذاته
Whipple's triad	Symptoms of hypoglycemia · documented low	True hypoglycemic disorder (not just a low number)	أعراض نقص سكر + سكر منخفض موثّق + تحسّن بعد إعطاء سكر ← يثبت إن الأعراض

	glucose · relief with glucose		فعلاً سببها نقص السكر مو شي تاني
Aortic dissection triad	Abrupt tearing chest/back pain · pulse or BP differential between limbs · widened mediastinum on CXR	Classic but insensitive — absence does not exclude dissection	ألم ممزّق مفاجئ + فرق بالنبض/الضغط بين الأطراف + اتساع المنصف بالأشعة ← كلاسيكي بس حساسيته منخفضة، غيابه ما بينفي التشخيص
Unhappy triad (knee)	ACL tear · MCL tear · meniscal injury	Valgus + rotational knee trauma	تمزق الرباط الصليبي الأمامي + الرباط الجانبي الإنسي + الغضروف الهلالي ← شائعة بإصابات الركبة الرياضية (دوران + ضغط جانبي)
Trousseau's syndrome	Migratory superficial thrombophlebitis	Occult malignancy (classically pancreatic)	التهاب وريدي سطحي متنقل من مكان لمكان ← ينذر بوجود ورم خبيث مخفي، خصوصًا بالبنكرياس
Meigs syndrome	Ovarian fibroma · ascites · pleural effusion	Resolves after tumor resection — mimics malignancy	ورم ليفي حميد بالمبيض + استسقاء بطني + انصباب جنبي ← يختفي بعد استئصال الورم، بيتلخبط مع السرطان المنتشر
Samter's triad	Asthma · nasal polyps · aspirin/NSAID sensitivity	AERD — avoid NSAIDs	ربو + سلائل أنفية + حساسية للأسبرين ومضادات الالتهاب ← لازم تجنّب هالأدوية تمامًا بهالمرضى
Kussmaul's sign	JVP rises (not falls) with inspiration	Constrictive pericarditis · tamponade · restrictive cardiomyopathy · RV infarct	ارتفاع ضغط الوريد الوداجي مع الشهيق بدل ما ينخفض (عكس الطبيعي) ← يدل على انضغاط القلب أو التهاب تامور مضيّق

02 Clinical Decision Rules & Scoring Systems

أدوات تقييم مخاطر مقنّنة — تُستخدم لتوجيه قرار "فحص إضافي أم لا" أو "قبول أم خروج".

Rule	Used for	Core components	Interpretation
Wells score (PE)	Pre-test probability of PE	Clinical DVT signs, PE = top Dx, HR>100, immobilization/surgery, prior VTE, hemoptysis, malignancy	<2 low / 2–6 moderate / >6 high; alternate 2-tier "PE likely/unlikely" at 4
PERC rule	Rules out PE in low pre-test probability patients *without* D-dimer	Age<50, HR<100, SpO_2≥95%, no hemoptysis, no estrogen, no prior VTE, no unilateral leg swelling, no recent surgery/trauma	All negative + low gestalt → PE effectively excluded
Wells score (DVT)	Pre-test probability of DVT	Active cancer, paralysis, bedridden≥3d/surgery, tenderness along veins, swelling, calf swelling>3cm, pitting edema, collateral veins, prior DVT, alt Dx as likely (−2)	≥2 DVT likely → ultrasound; <2 unlikely → D-dimer
HEART score	MACE risk in chest pain / possible ACS	History, ECG, Age, Risk factors, Troponin (0–2 each)	0–3 low risk / 4–6 moderate / 7–10 high
CURB-65	Community-acquired pneumonia severity / disposition	Confusion, Urea>7mmol/L, RR≥30, BP<90/60, Age≥65	0–1 outpatient / 2 consider admission / ≥3 consider ICU
Ottawa ankle rules	Need for ankle/foot X-ray after trauma	Pain in malleolar zone + bony tenderness at posterior tip/malleolus OR	Rule positive → imaging indicated; high sensitivity to safely skip films

		inability to bear weight 4 steps	
Ottawa knee rule	Need for knee X-ray after trauma	Age≥55, isolated patellar tenderness, fibular head tenderness, inability to flex 90°, inability to bear weight 4 steps	Any positive → imaging indicated
Canadian C-spine rule	Need for C-spine imaging in alert, stable trauma patients	High-risk factor present? → image. If not, low-risk factor allowing safe ROM testing? → test rotation 45° both ways	Inability to rotate or any high-risk factor → imaging
NEXUS criteria	Alternative C-spine clearance tool	No midline tenderness, no focal neuro deficit, normal alertness, no intoxication, no distracting injury	All absent → C-spine clinically cleared
Glasgow Coma Scale	Level of consciousness, trauma triage	Eye (4) + Verbal (5) + Motor (6)	13–15 mild / 9–12 moderate / ≤8 severe (consider airway protection)
qSOFA	Bedside sepsis-related deterioration flag (not a diagnostic test)	RR≥22, SBP≤100, altered mentation	≥2 → higher risk, prompts full sepsis work-up
PECARN head injury rule	Need for CT head in pediatric minor head trauma	Age-stratified: GCS, palpable skull fracture, AMS, scalp hematoma, LOC, mechanism, vomiting, headache	Negative in low-risk group → safe to observe without CT
Centor / McIsaac criteria	Probability of strep pharyngitis	Tonsillar exudate, tender anterior cervical nodes, fever, absence of cough (± age adjustment)	Higher score → consider testing/empiric treatment per local policy

ABCD2 score	Short-term stroke risk after TIA	Age≥60, BP≥140/90, Clinical features, Duration, Diabetes	Higher score → higher 2-day stroke risk, favors admission/urgent work-up
Alvarado score	Clinical probability of appendicitis	Migration, anorexia, nausea/vomiting, RLQ tenderness, rebound, fever, leukocytosis, left shift	Low scores argue against; high scores support surgical consultation
Glasgow-Blatchford score	Need for intervention in upper GI bleed	Urea, hemoglobin, SBP, HR, melena, syncope, liver disease, cardiac failure	Score 0 → safe for outpatient management; higher → endoscopy/admission
Rockall score	Mortality/rebleed risk in GI bleed (post-endoscopy)	Age, shock, comorbidity, endoscopic diagnosis, stigmata of recent hemorrhage	Higher score → higher mortality and rebleeding risk
SIRS criteria	Systemic inflammatory response (older sepsis-screening tool)	Temp>38 or <36, HR>90, RR>20, WBC>12k or <4k (≥2 of 4)	Non-specific; largely superseded by qSOFA/SOFA under Sepsis-3 but still tested historically
CHA_2DS_2-VASc	Stroke risk in atrial fibrillation	CHF, HTN, Age≥75(2pts), DM, prior stroke/TIA(2pts), vascular disease, Age65-74, female sex	Guides the decision to anticoagulate
HAS-BLED	Bleeding risk on anticoagulation	HTN, abnormal renal/liver function, prior stroke, bleeding history, labile INR, elderly, drugs/alcohol	Higher score flags closer monitoring — not an automatic reason to withhold anticoagulation
San Francisco Syncope Rule	Short-term serious-outcome risk after syncope	CHF history, Hct<30%, abnormal ECG, shortness of breath, SBP<90	Any positive → higher risk, consider admission/monitoring

PESI / sPESI	Mortality risk stratification in confirmed PE	Age, cancer, chronic cardiopulmonary disease, HR, SBP, RR, temperature, altered mentation, SpO_2	Low score (sPESI=0) → candidate for outpatient treatment

Exact cut-offs and point weights vary slightly between published versions — confirm against the version used in your exam curriculum before memorizing numbers verbatim.

03 Critical Time Windows

نوافذ زمنية حرجة — التأخر فيها بيغيّر النتيجة السريرية بشكل مباشر.

Scenario	Target window	Why it matters
STEMI — door to balloon (PCI-capable center)	≤90 min from first medical contact	Myocardial salvage; mortality rises with delay
STEMI — door to balloon (transfer needed)	≤120 min total	Beyond this, fibrinolysis is generally preferred if no contraindication
Ischemic stroke — IV thrombolysis (alteplase/tenecteplase)	Onset ≤4.5 h; door-to-needle goal ≤60 min	Benefit is highly time-dependent ("time is brain")
Ischemic stroke — mechanical thrombectomy	Up to 24 h in selected large-vessel-occlusion patients on advanced imaging	Extended window with perfusion-mismatch imaging selection
Sepsis — antibiotics	Within 1 h of recognition	Each hour of delay is associated with increased mortality
Trauma — "golden hour"	~first 60 min after major trauma	Conceptual target for definitive hemorrhage control, not an absolute cutoff
Testicular torsion — detorsion	<6 h for best salvage; salvage drops sharply after	Time-critical surgical emergency
Compartment syndrome — fasciotomy	Within ~6–8 h of onset	Irreversible muscle/nerve damage beyond this
Central retinal artery occlusion	Reversible window generally <90–100 min (rarely achievable in practice)	Ophthalmologic emergency; time-critical ischemia

04 Physical Exam Signs & Maneuvers

Sign	Elicited by	Suggests
Murphy's sign	Inspiratory arrest on RUQ palpation	Acute cholecystitis
McBurney's point tenderness	Palpation 1/3 from ASIS to umbilicus	Appendicitis
Rovsing's sign	LLQ palpation causes RLQ pain	Appendicitis
Psoas sign	Pain on hip extension / flexion against resistance	Retrocecal appendicitis, psoas irritation
Obturator sign	Pain on internal rotation of flexed hip	Pelvic appendicitis / abscess
Kernig's sign	Pain/resistance extending the knee with hip flexed	Meningeal irritation
Brudzinski's sign	Involuntary hip/knee flexion with neck flexion	Meningeal irritation
Cullen's sign	Periumbilical ecchymosis	Retroperitoneal/hemorrhagic pancreatitis, ruptured ectopic
Grey Turner's sign	Flank ecchymosis	Retroperitoneal hemorrhage, hemorrhagic pancreatitis
Kehr's sign	Referred left shoulder pain on supine positioning	Splenic rupture / diaphragmatic irritation
Battle's sign / raccoon eyes	Mastoid or periorbital ecchymosis	Basilar skull fracture
Tinel's sign	Tingling on percussion over median nerve at wrist	Carpal tunnel syndrome
Phalen's test	Symptoms reproduced by wrist flexion	Carpal tunnel syndrome
Chvostek's sign	Facial twitch on tapping facial nerve	Hypocalcemia

Trousseau's sign	Carpal spasm after BP-cuff inflation	Hypocalcemia (more specific than Chvostek's)
Homan's sign	Calf pain on forced dorsiflexion	Classically taught for DVT — low sensitivity/specificity, not relied on clinically

05 Toxidrome Recognition

أنماط تسمم سريرية للتعرف على التشخيص — بدون أسماء أدوية أو جرعات ترياق (مستثناة حسب الطلب).

Toxidrome	Pupils	Vitals pattern	Other features	Common causative agents
Opioid	Miosis (pinpoint)	Bradypnea, hypotension, bradycardia	CNS depression, decreased bowel sounds	Morphine, fentanyl, heroin, pethidine (meperidine), tramadol, codeine
Anticholinergic	Mydriasis	Tachycardia, hyperthermia	Mad as a" hatter, blind as a bat, red as a beet, hot as a hare, dry as a bone" — delirium, dry flushed skin, urinary retention	Atropine, scopolamine, diphenhydramine & other antihistamines, tricyclic antidepressants, datura (jimsonweed) plant poisoning
Cholinergic	Miosis	Bradycardia (can be tachycardic)	SLUDGE/DUMBELS — salivation, lacrimation, urination, diarrhea, GI distress, emesis; bronchorrhea, miosis, weakness	Organophosphate pesticides, carbamate pesticides, nerve agents (sarin, VX), physostigmine, some muscarine-containing mushrooms
Sympathomimetic	Mydriasis	Tachycardia, hypertension, hyperthermia	Agitation, diaphoresis (moist skin — key differentiator from anticholinergic)	Cocaine, amphetamines, methamphetamine, synthetic cathinones ("bath salts"), high-dose

				pseudoephedrine/caffeine
Sedative-hypnotic	Normal to small, reactive	Mild respiratory/CNS depression	Slurred speech, ataxia, normal-reactive pupils (distinguishes from opioid)	Benzodiazepines, barbiturates, ethanol, zolpidem/"Z-drugs", GHB
Serotonin syndrome	Mydriasis	Hyperthermia, autonomic instability	Clonus, hyperreflexia, tremor — lower extremities > upper; acute onset	SSRIs, SNRIs, MAOIs, tramadol, MDMA, linezolid — especially when combined
Neuroleptic malignant syndrome	Normal	Hyperthermia	"Lead-pipe" rigidity, bradyreflexia, gradual onset over days (contrast with serotonin syndrome)	Typical & atypical antipsychotics (e.g. haloperidol); abrupt withdrawal of dopaminergic agents (e.g. levodopa)

> Agents listed are the classically cited examples for exam purposes, not an exhaustive toxicology list — dosing, antidotes and management are out of scope here (see the excluded-topics note on the cover).

06 Laboratory Values & Critical Thresholds

قيم مخبرية حرجة شائعة بالامتحانات — القيم الدقيقة بتختلف حسب المختبر، فخذها كمرجع تقريبي وتأكد من مرجعية مختبرك.

Parameter	Critical / actionable range	Clinical note
Sodium	<120 or >160 mmol/L	Rapid correction of chronic hyponatremia risks osmotic demyelination
Potassium	<2.5 or >6.5 mmol/L	>6.5 or ECG changes → emergent treatment
Glucose	<40 or >500 mg/dL	Correlate with mental status and ketones/osmolality
Lactate	>2 mmol/L abnormal; >4 mmol/L severe	Marker of tissue hypoperfusion; trend matters more than a single value
Anion gap	>12 (assay-dependent, ~8–16 normal)	Elevated → think MUDPILES differential (see mnemonics)
Hemoglobin	<7 g/dL commonly cited transfusion trigger (context-dependent)	Threshold adjusted for cardiac disease, active bleeding, symptoms
Platelets	<20,000–50,000/µL spontaneous bleeding risk rises	Procedure-specific thresholds differ
INR	>1.5 often used as coagulopathy threshold	Therapeutic range for most indications is 2–3; mechanical valves may target higher
Troponin	Assay- and 99th-percentile-dependent	Serial trend (delta) matters more than a single absolute value
pH (arterial)	<7.2 or >7.6 critical	Interpret with pCO_2 and bicarbonate together

07 Diagnostic Criteria & Clinical Definitions

تعريفات رسمية لحالات طوارئ شائعة — كل وحدة منها مبنية على معايير محددة لازم تُحفظ بالترتيب.

Condition	Defining criteria
Sepsis-3	Suspected/confirmed infection + acute increase in SOFA score ≥2 points
Septic shock (Sepsis-3)	Sepsis + vasopressors required to maintain MAP≥65 + lactate>2 mmol/L despite adequate fluid resuscitation
DKA	Glucose>250 mg/dL + pH<7.3 + bicarbonate<18 mEq/L + positive ketones (can occur with near-normal glucose — "euglycemic DKA," especially with SGLT2 inhibitor use)
HHS (Hyperosmolar Hyperglycemic State)	Glucose>600 mg/dL + serum osmolality>320 mOsm/kg + minimal ketosis, pH generally >7.3
Berlin definition — ARDS	Onset within 1 week of insult + bilateral opacities not fully explained by effusion/collapse/nodules + respiratory failure not fully explained by cardiac failure/fluid overload + PaO_2/FiO_2 ratio defines mild/moderate/severe
HELLP syndrome	Hemolysis + Elevated Liver enzymes + Low Platelets, in the context of pregnancy (variant of severe preeclampsia)
Preeclampsia	BP≥140/90 after 20 weeks gestation + proteinuria OR new end-organ dysfunction (even without proteinuria)
Eclampsia	Preeclampsia + new-onset tonic-clonic seizures not attributable to another cause
AKI (KDIGO)	Rise in creatinine ≥0.3 mg/dL within 48h, or ≥1.5× baseline within 7 days, or urine output <0.5 mL/kg/h for 6h

08 Red Flags / Must-Not-Miss Presentations

علامات إنذارية بحسب الشكوى الرئيسية — وجودها بيستدعي تقييم إضافي قبل أي تسريح.

Headache

- Thunderclap onset
- "Worst headache of life"
- Focal neuro deficit
- Fever + neck stiffness
- New headache age>50 or in pregnancy/postpartum
- Headache with Valsalva/exertion

Diagnoses to exclude: subarachnoid hemorrhage, meningitis/encephalitis, intracranial mass/tumor, temporal arteritis in older adults, cerebral venous sinus thrombosis (especially in pregnancy/postpartum).

Chest pain

- Tearing/ripping pain radiating to back
- Pulse/BP differential between arms
- Syncope with chest pain
- Pleuritic pain + risk factors for PE
- Pain with diaphoresis/dyspnea at rest

Diagnoses to exclude: aortic dissection, pulmonary embolism, myocardial infarction, tension pneumothorax.

Abdominal pain

- Pain out of proportion to exam
- Rigid/peritonitic abdomen
- Pulsatile mass / syncope (ruptured AAA)
- Pregnancy + abdominal pain + bleeding
- Pain + hemodynamic instability

Diagnoses to exclude: ruptured abdominal aortic aneurysm, mesenteric ischemia, ruptured ectopic pregnancy, viscus perforation.

Back pain

- Saddle anesthesia / bowel-bladder dysfunction
- Progressive motor weakness
- Fever + IV drug use history
- Unexplained weight loss / cancer history
- Age>50 with new-onset severe pain

Diagnoses to exclude: cauda equina syndrome, epidural abscess/osteomyelitis, abdominal aortic aneurysm/dissection, spinal metastatic disease.

Dyspnea / shortness of breath

- Stridor or inability to speak full sentences
- Silent chest in a known asthmatic
- Unilateral absent breath sounds + hypotension
- Sudden pleuritic pain + VTE risk factors
- Cyanosis or SpO_2 refractory to oxygen

Diagnoses to exclude: tension pneumothorax, pulmonary embolism, upper airway obstruction, critical asthma exacerbation — a "silent chest" is an ominous sign, not a sign of improvement.

Syncope

- Syncope during exertion
- Chest pain or palpitations preceding the event
- Abnormal ECG (long QT, Brugada, WPW, high-grade block)
- Family history of sudden cardiac death
- No prodrome in an older adult

Diagnoses to exclude: malignant cardia arrhythmia, aortic stenosis, myocardial infarction — distinguished from benign vasovagal syncope by the absence of prodrome and the link to exertion.

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09 Mnemonics

اختصارات ذاكراتية أساسية للتاريخ المرضي، التقييم الأولي، وتشخيص الحالات التفريقية.

Mnemonic	Expansion	Used for	شرح مختصر
SAMPLE	Symptoms, Allergies, Medications, Past history, Last meal, Events	Rapid focused history	تاريخ مرضي سريع بالطوارئ: الأعراض، الحساسية، الأدوية، الأمراض السابقة، آخر وجبة، ظروف الحادثة
AMPLE	Allergies, Medications, Past history, Last meal, Events	Trauma-focused history	نفس فكرة SAMPLE بس للمريض الرضحي تحديدًا، مع التركيز على ظروف الحادثة
OPQRST	Onset, Provocation/Palliation, Quality, Radiation, Severity, Timing	Characterizing pain/symptoms	لوصف أي ألم بالتفصيل: وقت البداية، المحفّز والمخفّف، نوع الألم، مكان انتشاره، شدته، وتوقيته
ABCDE	Airway, Breathing, Circulation, Disability, Exposure	Primary survey / resuscitation sequence	ترتيب أولويات التقييم والإنعاش: مجرى الهواء، التنفس، الدورة الدموية، الوعي، ثم كشف المريض بالكامل
5 H's & 5 T's	Hypovolemia, Hypoxia, H^+(acidosis), Hypo/hyperkalemia, Hypothermia · Tension pneumothorax, Tamponade, Toxins, Thrombosis (coronary/pulmonary), Trauma	Reversible causes of cardiac arrest (PEA/asystole)	الأسباب القابلة للعلاج بتوقف القلب اللانبضي — خمسة أسباب استقلابية وخمسة أسباب ميكانيكية/سببية
AEIOU TIPS	Alcohol, Epilepsy/Endocrine, Insulin, Overdose, Uremia · Trauma, Infection, Psychiatric,	Differential for altered mental status	تشخيص تفريقي منظّم لأي مريض بتغيّر مفاجئ بمستوى الوعي

	Stroke/Space-occupying lesion		
MUDPILES	Methanol, Uremia, DKA, Propylene glycol/Paraldehyde, Isoniazid/Iron, Lactic acidosis, Ethylene glycol, Salicylates	Elevated anion-gap metabolic acidosis differential	أسباب الحماض الاستقلابي مع فجوة أنيونية مرتفعة — تُستخدم لما تلاقي فجوة أنيونية زايدة بتحليل الغازات
FAST	Face drooping, Arm weakness, Speech difficulty, Time to call	Public stroke-recognition screen	أداة توعوية شعبية بسيطة لاكتشاف السكتة الدماغية مبكرًا والاتصال بالطوارئ فورًا

10 Examination Pearls & Diagnostic Traps

فخاخ شائعة بالامتحانات — نقاط بتتكرر غلطها بين المتقدمين للبورد.

First step ≠ definitive step. Exams frequently test "most appropriate next step," not final treatment — e.g. in suspected cauda equina, the next step is urgent MRI, not immediate surgery.

Absence of a classic sign does not exclude the diagnosis. The aortic dissection triad, Murphy's sign, and Kernig's/Brudzinski's signs all have limited sensitivity — a negative exam does not rule out the disease.

Homan's sign is a historical distractor. It is low-yield clinically (poor sensitivity/specificity for DVT) but still appears as a tempting wrong answer on written exams.

qSOFA is a bedside flag, not a sepsis diagnostic tool. Sepsis-3 is defined by SOFA, not qSOFA — a common examiner trap.

Serotonin syndrome vs. NMS. Fast onset + hyperreflexia/clonus favors serotonin syndrome; slow onset over days + lead-pipe rigidity + bradyreflexia favors NMS.

Euglycemic DKA exists. Normal/near-normal glucose does not exclude DKA, especially with SGLT2-inhibitor use — a frequently tested exception to the "DKA needs high glucose" assumption.

Ottawa rules are rule-out tools, not rule-in tools. A negative rule means imaging can safely be skipped — it does not prove a fracture is absent elsewhere in the limb.

PERC only applies to patients already judged low-probability. Using PERC on a patient the clinician already suspects has PE (moderate/high gestalt) is a common misuse — it is a gestalt-dependent rule, not a universal PE rule-out test.

Beck's triad is present in only a minority of tamponade cases. Exam questions overstate its sensitivity — don't wait for all three signs before considering bedside echo in a hypotensive patient with risk factors.

Testicular torsion can present as isolated abdominal pain in prepubertal boys. Always examine the genitalia in a young male with unexplained abdominal pain — a classic missed-diagnosis trap.

A normal ECG does not exclude ACS. Posterior MI and early ischemia can show a deceptively unremarkable 12-lead; serial ECGs and troponin trends matter more than a single reading.

"Worst headache of life" with a normal CT isn't the end of the work-up. CT sensitivity for subarachnoid hemorrhage falls after roughly 6 hours from symptom onset — lumbar puncture or

CT angiography may still be indicated per local protocol.

11 Communication & Professional Frameworks

أطر تواصل منظّمة — جزء أساسي من كفايات الامتحان الشفهي و OSCE التواصلي (هون فقط الإطار، بدون سيناريوهات Oral Board المستثناة).

Framework	Steps	Used for	شرح مختصر
SPIKES	Setting, Perception, Invitation, Knowledge, Emotions, Strategy/Summary	Breaking bad news	ترتيب منظّم لإيصال خبر صعب: تجهيز المكان، معرفة فهم المريض الحالي، أخذ إذنه قبل التفاصيل، إعطاء المعلومة بوضوح، التعامل مع رد فعله العاطفي، ثم تلخيص الخطة التالية
SBAR	Situation, Background, Assessment, Recommendation	Structured clinical handover	طريقة منظّمة لتسليم الحالة بين الفريق الطبي: وش الوضع الحالي، الخلفية المرضية، تقييمك، وتوصيتك — تقلل فقدان المعلومات أثناء التسليم
Ask-Tell-Ask	Assess understanding → share information → check understanding	Shared decision-making, informed consent	تسأل المريض وش فاهم، تشرحله المعلومة، وبعدين تتأكد إنه فعلاً استوعب — تضمن تواصل فعّال بدل تلقين معلومات بس
NURSE statements	Naming, Understanding, Respecting, Supporting, Exploring emotion	Responding to emotion in difficult conversations	جمل جاهزة للتعامل مع مشاعر المريض وقت الأزمة: تسمية الشعور، إظهار الفهم، احترام موقفه، تقديم الدعم، واستكشاف مشاعره أكثر